

ENCOUNTER PROGRESS NOTE

PATIENT

Name: Bernard Ostrowski
DOB: 1950-02-14
MRN: MRN-2026-0412
Date of Service: 2026-04-22
Provider: Lakeside Multi-Specialty Associates
Visit Type: Office / Outpatient

CHIEF COMPLAINT

3-day history of acute worsening of cough, increased purulent sputum, and dyspnea at rest.

HISTORY OF PRESENT ILLNESS

Mr. Ostrowski is a 76-year-old male with established COPD (50 pack-year smoking history, on home oxygen 2 L NC) who presents with a 3-day acute exacerbation of his baseline shortness of breath. He reports increased cough with thick yellow-green sputum, low-grade fever to 100.8 degF at home, and inability to walk to his mailbox without stopping. He has used his albuterol rescue inhaler every 2 hours without relief.

PAST MEDICAL HISTORY

Severe COPD (GOLD stage 3) on home O2, prior smoker (quit 2018), hypertension, hyperlipidemia.

PHYSICAL EXAMINATION

Temp 100.6 degF, BP 132/78, HR 104, RR 24, SpO2 88% on 2L NC (baseline 92% on 2L). Diffuse expiratory wheezes with prolonged expiratory phase. Scattered rhonchi bilaterally, more pronounced at the right lung base. Use of accessory muscles. No peripheral edema.

LABS / IMAGING

Chest X-ray (CPT 71046): focal right lower lobe consolidation consistent with pneumonia, superimposed on chronic hyperinflation and flattened diaphragms. CBC: WBC 14.2 with left shift. Pulmonary function trended: documented baseline FEV1 38% predicted (severe obstruction) confirming underlying COPD.

ASSESSMENT

- Acute exacerbation of severe COPD (J44.1). Documented baseline obstructive disease with acute worsening of dyspnea, increased sputum production, and hypoxia requiring treatment escalation beyond baseline therapy.
- Community-acquired pneumonia, right lower lobe (J18.9), confirmed by chest X-ray.

PLAN

Prednisone 40 mg PO daily x 5 days. Levofloxacin 750 mg PO daily x 7 days (covers CAP plus COPD exacerbation pathogens). Increase home O2 to 3 L NC. Albuterol/ipratropium nebs Q4H. Follow-up in 7 days; return precautions reviewed.

PROVIDER SIGNATURE

Electronically signed by Dr. Elena Vasquez, MD on 2026-04-22.